CONFIDENTIAL ETHICAL AND LEGAL DEFENSE MEMORANDUM

Supplement to OSC Case No. MA-25-004957

To:
U.S. Office of Special Counsel (OSC)
1730 M Street, N.W., Suite 218
Washington, D.C. 20036-4505

From:
Kevin D. Turnage, RN
Policy and Procedure Officer
Phoenix Indian Medical Center

Subject: Protected Disclosures and Retaliation – Supplement to Pending OSC Case

Date: September 26, 2025

I. INTRODUCTION

This memorandum is submitted as a formal supplement to OSC Case No. MA-25-004957, pursuant to the Whistleblower Protection Act (5 U.S.C. § 2302). It provides further documentation and legal analysis of the retaliation I am currently experiencing as a result of making protected disclosures in my capacity as a federal nurse at the Phoenix Indian Medical Center (PIMC).

I respectfully request that the Office of Special Counsel (OSC) incorporate this memorandum into the record of the above-referenced case, conduct further investigation, and take prompt action to protect my federal employment rights and professional license.

II. NATURE OF PROTECTED DISCLOSURES

In my official capacity as Policy and Procedure Officer at PIMC, I submitted protected disclosures through appropriate federal channels that addressed:

1. Unsafe clinical and administrative conditions adversely affecting Indigenous and cross-border tribal populations;
2. Noncompliance with federal health care policy and ethical standards by executive leadership;
3. Suppression and retaliation against staff raising concerns about patient safety, access to care, and health disparities;
4. Manipulation of policy procedures and governance protocols in a manner inconsistent with IHS directives;
5. Neglect of vulnerable patients' rights under federal and tribal health mandates.

These disclosures were lawful, made in good faith, and are protected under the Whistleblower Protection Act (WPA).

III. RETALIATORY INQUIRY: SHAM PROCESS, CONFLICTS OF INTEREST, AND TARGETING OF PROTECTED DISCLOSURES

On September 25, 2025, I was subjected to an administrative inquiry ostensibly initiated in response to a "hostile work environment" complaint submitted by Clinton Pope, Chief Quality Officer. However, the true focus of the inquiry was my protected whistleblower disclosures, not any legitimate workplace misconduct.

A. Subject of the Inquiry: Protected Disclosures

The questions posed during the inquiry targeted:

- Communications I made to federal leadership and oversight agencies;
- Objections I raised to the suppression of ethical nursing standards;
- My drafting and editing of internal policy documents;
- Use of approved workplace tools such as Grammarly;
- Statements I made regarding governance failures and health equity violations.

This inquiry did not concern unlawful conduct, but rather attempted to reframe protected communications as disruptive behavior. Under 5 U.S.C. § 2302(b)(8), such retaliatory inquiries are prohibited personnel practices.

B. Conflict of Interest – Naomi Hixson

The inquiry was led by Naomi Hixson, PIMC's Compliance Officer. Ms. Hixson is a material witness to the very communications at issue, having been a direct recipient of several emails referenced in the complaint.

Her involvement represents a clear conflict of interest, violating principles of due process and rendering the inquiry procedurally defective.

C. Pretextual Use of Complaint Mechanisms

The administrative inquiry originated from a complaint filed by Mr. Pope—an individual whose conduct I had previously challenged in protected disclosures. This complaint was used as a

pretext to initiate an investigation against me, targeting my speech rather than addressing any actionable misconduct.

Such misuse of internal complaint mechanisms to retaliate against a whistleblower constitutes a systemic failure and legal violation.

IV. ETHICAL DUTIES AND PROFESSIONAL OBLIGATIONS

As a Registered Nurse licensed in Arizona and a federal employee, I am legally and professionally required to raise concerns about unsafe, unethical, or unlawful practices.

A. Whistleblower Protection Act (5 U.S.C. § 2302(b)(8))

Protects federal employees who disclose:

- Legal or regulatory violations;
- Gross mismanagement or waste;
- Abuse of authority;
- Substantial danger to public safety or health.

B. Arizona Nurse Practice Act (A.R.S. § 32-1664; § 32-1601(16))

Requires licensed nurses to report unethical, incompetent, or illegal behavior, and protects those who do so in good faith.

C. ANA Code of Ethics for Nurses (2025)

Key provisions applicable to my case include:

- Provision 3.5 – Duty to report unethical or unsafe practices;
- Provision 4 – Accountability and personal integrity in nursing practice;
- Provision 6 – Responsibility to create and sustain ethical work environments;
- Provision 8 – Advocacy to eliminate health disparities;
- Provision 10 – Obligation to protect vulnerable populations, including Indigenous and transnational communities.

V. ORGANIZATIONAL CONTEXT

Phoenix Indian Medical Center serves American Indian and Alaska Native populations across:

- Arizona, Nevada, Utah, and tribal jurisdictions in Mexico;
- Fort McDowell Yavapai Nation;
- San Lucy District of the Tohono O'odham Nation;
- Tonto Apache Tribe;
- Yavapai-Apache Indian Tribe;
- Yavapai-Prescott Indian Tribe;
- Tribal members from other parts of the United States receiving care in Phoenix.

My protected disclosures addressed deficiencies in care delivery, governance, and patient protection affecting these underserved populations.

VI. REQUEST FOR OSC ACTION

I respectfully request that the U.S. Office of Special Counsel:

1. Immediately intervene to suspend the ongoing administrative inquiry, which is clearly retaliatory and compromised by conflicts of interest;
2. Initiate a formal investigation into retaliatory conduct by leadership at Phoenix Indian Medical Center, including:
    - Debra Louise Ward Lund, Chief Executive Officer and licensed Registered Nurse;
    - Naomi Hixson, Compliance Officer and conflicted investigator;
    - Clinton Pope, Chief Quality Officer and complainant in the pretextual inquiry;
    - Kathy Dill, Chief Operating Officer, who failed to mitigate or prevent retaliatory activity;
    - Any other executive officials who were aware of, involved in, or failed to act against the retaliatory conduct;
3. Affirm my protected status as a whistleblower under 5 U.S.C. § 2302 and ensure no further adverse personnel actions are taken;
4. Recommend corrective or disciplinary action against responsible officials for misuse of authority and violation of federal law;
5. Ensure that Indian Health Service (IHS) and its leaders uphold the rights of federal employees to report wrongdoing without fear of retaliation, in accordance with applicable law and ethical obligations.

VII. STATUS OF SUPPORTING MATERIALS

I currently possess:

- The written inquiry questions posed during the administrative proceeding on September 25, 2025;
- Two affidavit-style summaries documenting portions of that session.

However, I have not yet had the opportunity to review or verify the accuracy of those affidavits. I expressly reserve all rights to amend, supplement, or correct this submission once I have conducted a full review.

Additional materials are available upon request, including:

- Email communications;
- Policy drafts;
- Ethical objections and internal reports;
- Correspondence with IHS and external oversight entities.

VIII. CONCLUSION

My actions have complied with all applicable legal, professional, and ethical duties. I have acted in good faith to report misconduct, protect patients, and uphold federal standards of care and governance. The retaliation I am now experiencing is not only unlawful but structurally corrosive to the mission of Indian Health Service and the principles of public health.

I respectfully ask that the Office of Special Counsel incorporate this memorandum into OSC Case No. MA-25-004957, investigate the facts herein, and take immediate corrective action to halt further retaliation and ensure accountability.

Respectfully submitted,

Kevin D. Turnage, RN